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Preferred name: Ashley Anne Lusk (Legal name: Ashley Anne Lusk) | DOB: 9/4/1965 | MRN: 95409491

PCP: Erica T Ting, MD



3/12/2026

Re: Primary letter of support for gender affirming surgery for Ashley

DOB: 9/4/1965

Consultation Date: 3/27/2026

Consultation Provider: Bonapace-Potvin, Michelle, MD

Consultation Facility: MHealth Fairview

To Whom It May Concern,

My name is Erica T Ting, MD and I am a licensed physician by the Minnesota Board of Medical Practice. I have been independently licensed for 4 years and have been working with people experiencing gender dysphoria since 2022.

- I am competent in the assessment and treatment of gender dysphoria and gender incongruence.
- I am competent in the assessment, diagnosis, and treatment of co-existing mental health disorders such as mood disorders, personality disorders, and substance related disorders and the ability to distinguish these from gender dysphoria.
- I am competent in the assessment of a patient's capacity to consent for treatment.
- I have opportunities to engage with other health care professionals from different disciplines within the field of transgender health for consultation and referral, as needed.
- I will be available for coordination of care should any mental health concerns arise during or after the gender-affirming procedure process.

Ashley Anne Lusk (legal name is Ashley Anne Lusk) is a 60 y.o. female who first came to see me on 2/14/2025 for care related to gender incongruence and gender dysphoria. I have met with this patient 5 times since for gender-affirming medical care and gender-affirming hormone therapy.

- Ashley has been diagnosed with gender dysphoria based on the criteria in the Diagnostic and Statistical Manual of Mental Health Disorders, fifth edition text revision (DSM 5-TR).
- Ashley has displayed marked and sustained incongruence between her experienced/expressed gender and assigned gender for at least 6 months' duration as manifested by: marked incongruence between one's

experienced/expressed gender and primary and/or secondary sex characteristics , strong desire to be rid of one's primary and/or secondary sex characteristics because of a marked incongruence with one's experienced/expressed gender , strong desire for the primary and/or secondary sex characteristics of another gender, strong desire to be another gender, strong desire to be treated as another gender, and strong conviction that one has the typical feelings and reactions of another gender

- Ashley's gender dysphoria is associated with clinically significant distress or impairment in social, occupational, or other critical areas of functioning.
- per the diagnostic assessment, Ashley does not meet criteria for other mental health diagnoses at this time and mental health appears to be stable and reasonably well-controlled.
- other possible causes of apparent gender incongruence have been identified and excluded.
- her options and plans for social transition have been discussed.
- she has been on gender-affirming hormone therapy since 2022.
- Ashley has been living full-time in her identified gender since 2022.

Ashley currently meets the World Professional Association for Transgender Health (WPATH) Standards of Care for gender-affirming surgery of facial feminization surgery.

This procedure/intervention is considered a medically necessary treatment by the WPATH Standards of Care.

- Ashley is pursuing facial feminization surgery to better align her gender identity with her physical body.
- Ashley demonstrates emotional and cognitive maturity to make informed decisions about her healthcare and has educated herself about the risks and benefits of facial feminization surgery.
 - specifically, Ashley is aware of the effects of this procedure/intervention on reproduction and has explored reproductive options.
 - moreover, Ashley is aware that some procedures such as chest and genital surgery are considered "irreversible" by the WPATH Standards of Care.
- Ashley has the capacity to consent to facial feminization surgery. Informed consent will be obtained by the clinician/surgeon who is performing this procedure/intervention.

I do not have any concerns about Ashley and I support her decision to pursue medically necessary gender-affirming procedures/interventions at this time. There are no significant mental health conditions that would negatively affect this procedure/intervention at this time.

In some cases, denial of access to this procedure/intervention can lead to increased isolation, decreased work performance, and decreased sexual and interpersonal functioning. Gender-affirming procedures/interventions are expected to alleviate dysphoria and increase comfort with self.

If you have any questions or need any additional information, please do not hesitate to contact me at 9529938000.

Sincerely,



Erica T Ting, MD